

DISCHARGE SUMMARY

Discharge Date: June 4, 2026

Thornton, Gerald R.

MRN	LC-887342	DOB	07/05/1946	Age / Sex	79 M / Male
Admitted	May 31, 2026	Discharged	June 4, 2026	Attending	Dr. Aisha Okonkwo, MD — Pulmonology

DIAGNOSES

Admitting Diagnosis:
Acute exacerbation of COPD with hypercapnic respiratory failure

- Discharge Diagnosis / Diagnoses:**
- Acute exacerbation of COPD (GOLD stage 3), now resolving
 - Hypercapnic respiratory failure — resolved
 - Community-acquired pneumonia as likely precipitant
 - Atrial fibrillation with rapid ventricular response — rate-controlled
 - Malnutrition, moderate

HISTORY OF PRESENT ILLNESS

Mr. Thornton is a 79-year-old male with severe COPD (GOLD stage 3, FEV1 38% predicted) and a 55-pack-year smoking history (quit 2011) who presented to the ED in moderate respiratory distress with worsening dyspnea, productive cough with yellow-green sputum, and fever to 38.7°C over 3 days. SpO2 on room air was 82%. ABG on presentation: pH 7.28, pCO2 68, pO2 54 on room air. CXR showed right lower lobe consolidation consistent with pneumonia. He was placed on BiPAP and admitted to a monitored bed.

HOSPITAL COURSE

Respiratory: BiPAP initiated in ED and continued for 18 hours. Transitioned to high-flow nasal cannula, then weaned to 2L NC by day 3. Discharged on home O2 2L NC at rest (pre-existing). Repeat ABG prior to discharge: pH 7.40, pCO2 52, pO2 68.

Infection: Sputum culture growing Haemophilus influenzae — sensitive to amoxicillin-clavulanate. Completed 5-day course of IV ampicillin-sulbactam; transitioned to oral amoxicillin-clavulanate at discharge for an additional 2 days to complete a 7-day total antibiotic course.

Bronchodilation: Nebulized ipratropium/albuterol Q4H during acute phase. Transitioned to MDI regimen at discharge. Inhaler technique assessed and re-educated by respiratory therapy.

Atrial Fibrillation: New rapid AF on telemetry — rate up to 148 bpm. Rate controlled with IV metoprolol and transitioned to oral. Anticoagulation initiated with apixaban after discussion of stroke risk (CHA2DS2-VASc score 5). Cardiology consult obtained.

Nutrition: Moderate malnutrition noted — BMI 18.1. Dietitian consult completed. High-protein supplements initiated. Referred to outpatient nutrition support.

PAST MEDICAL HISTORY

- COPD, GOLD stage 3 — on home O2 2L NC at rest
- 55-pack-year smoking history, quit 2011
- Hypertension
- Osteoporosis — on alendronate
- Prior COPD exacerbations x3 in last 2 years

DISCHARGE MEDICATIONS

MEDICATION / DOSE	ROUTE / FREQ	NOTES
Amoxicillin-clavulanate 875/125 mg	PO BID x 2 days	Complete antibiotic course — final 2 days only.
Prednisone 40 mg	PO daily x 3 more days	Continue taper then stop. Day 4–6 of 7-day course.
Tiotropium 18 mcg	Inhaled once daily	Continue. Confirm correct HandiHaler technique.
Fluticasone/salmeterol 500/50	Inhaled BID	Continue ICS/LABA.
Albuterol MDI	2 puffs q4–6h PRN	Rescue inhaler — use as needed.
Apixaban 5 mg	PO BID	NEW — atrial fibrillation. Do not stop without cardiology guidance.
Metoprolol succinate 25 mg	PO daily	NEW — rate control for AF.
Alendronate 70 mg	PO weekly (fasting)	Continue home dose.

■ PENDING ITEMS — PCP ACTION REQUIRED

- Blood cultures x2 drawn on admission — finalized as no growth. No further action.
- Echocardiogram ordered by cardiology — not yet scheduled. Confirm appointment at cardiology follow-up.
- Spirometry (PFTs) overdue — last performed 18 months ago. Order at pulmonology visit.
- Apixaban NEW for new-onset AF — ensure patient has prescription and understands not to stop.

FOLLOW-UP PLAN

SPECIALTY	FOLLOW-UP DATE	INSTRUCTIONS
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Pulmonology	June 11, 2026	Post-exacerbation assessment. Review culture results. Assess for pulmonary rehab enrollment.
Cardiology	June 17, 2026	New AF management, anticoagulation review, consider rhythm vs rate strategy, echocardiogram.
Primary Care	June 9, 2026	Medication reconciliation, BP check, ensure antibiotic course completed, nutrition follow-up.
Pulmonary Rehab	June 23, 2026	Initial evaluation — referral placed at discharge. Call (555) 312-7700 to confirm enrollment.

DISPOSITION & INSTRUCTIONS

Discharged home with VNA (visiting nurse) services arranged for the first week. Home O2 at 2L NC — patient educated on oxygen safety (no smoking, ventilated space). COPD action plan reviewed: increase albuterol use, begin oral steroids if prescribed, and call pulmonology or go to ED if SpO2 drops below 88% or dyspnea does not improve within 24 hours. Patient discharged in stable but guarded condition with significant medical complexity requiring close follow-up.

Electronically signed by: **Aisha Okonkwo**, MD, FCCP — Pulmonology, Lakeview Community Hospital

Date/Time: June 4, 2026 | Lakeview Community Hospital

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